

In severe vulvovaginal candidiasis, single-dose treatment with clotrimazole 500-mg vaginal tablets may not be effective. Longer treatment with the 100- or 200-mg vaginal tablets is recommended.

Usual pediatric dose

Children up to 12 years of age: Safety and efficacy have not been established.

SYMPTOMS AND TREATMENT FOR OVERDOSE AND ANTIDOTE(S):

No data available.

PACKING/PACK SIZES:

Strip pack of 10's

STORAGE CONDITIONS, USER INSTRUCTIONS AND PHARMACEUTICALS PRECAUTIONS:

Store below 30°C.

SHELF LIFE: 3 years.

NAME AND ADDRESS OF MANUFACTURER AND DISTRIBUTOR IN MALAYSIA:

SM PHARMACEUTICALS SDN BHD (218620-M)

Lot 88, Sungai Petani Industrial Estate

08000 Sungai Petani

Kedah Darul Aman, Malaysia.

24.02.2009

SM PHARMACEUTICALS SDN. BHD.

CLOZOLE (Clotrimazole) Pessary 100, 200 and 500 mg

DESCRIPTION:

A white to off white almond shape pessary.

COMPOSITION:

Each pessary contains Clotrimazole 100, 200 and 500 mg.

ACTIONS AND MODE OR MECHANISMS OF ACTION:

Fungistatic; may be fungicidal, depending on concentration; exact mechanism of action is unknown. It inhibits biosynthesis of ergosterol or other sterols, damaging the fungal cell wall membrane and altering its permeability. As a result, loss of essential intracellular elements may occur.

It also inhibits biosynthesis of triglycerides and phospholipids by fungi. In addition, it inhibits oxidative and peroxidative enzyme activity, resulting in intracellular buildup of toxic concentrations of hydrogen peroxide, which may contribute to deterioration of subcellular organelles and cellular necrosis. In *Candida albicans*, inhibits transformation of blastopores into invasive mycelial form.

PHARMACOLOGY (SUMMARY OF PHARMACODYNAMICS AND PHARMACOKINETICS)

Absorption: 3 to 10% estimated to be absorbed following intravaginal administration. It is poorly absorbed from the gastro intestinal tract. It is metabolised in the liver to inactive compounds and excreted in the faeces and urine. When applied topically clotrimazole penetrates the epidermis but there is little if any systemic absorption. Slight absorption has been reported following vaginal administration.

Biotransformation: Rapidly metabolized to inactive metabolites.

INDICATIONS:

Vaginitis caused by fungi mostly *Candida* and / or *Trichomonas*. Super-infections caused by clotrimazole-sensitive bacteria.

CONTRAINDICATIONS:

Hypersensitivity to clotrimazole. Allergy to azoles.

SIDE EFFECTS / ADVERSE REACTIONS:

Gastro-intestinal disturbance, elevation of liver enzymes, and mental depression have been reported after administration of clotrimazole by mouth. Local reactions including irritation and burning may occur in patients treated topically; contact allergic dermatitis has been reported.

PRECAUTIONS / WARNINGS:

Breast feeding:

It is not known whether vaginal azoles are distributed into breast milk. However, problems in humans have not been documented.

Pediatrics:

No information is available on the relationship of age to the effects of vaginal azoles in pediatric patients. Safety and efficacy have not been established in children up to 12 years of age.

Geriatrics:

Appropriate studies on the relationship of age to the effects of vaginal azoles have not been performed in the geriatric population. However, no geriatrics-specific problems have been documented to date.

PREGNANCY / LACTATIONS:

Experimental and clinical investigations give no indication that Clotrimazole has any harmful effects on the mother and child when administered during pregnancy.

Sanitation of the birth canal should be ensured particularly during the last 4-6 weeks of pregnancy.

During pregnancy, Clotrimazole should be inserted without using an applicator.

Administration during the 1st trimester of pregnancy should be used with clear indication.

DRUG INTERACTIONS:

There are no known physical incompatibilities or drug interactions between Clotrimazole pessary and other drugs.

Antagonism between clotrimazoles and polyene antibiotics in vitro has been reported. This may be because imidazoles inhibit ergosterol synthesis whereas polyene antibiotics exert their antifungal action by binding to cell membrane sterols.

Induction of hepatic microsomal enzymes by clotrimazole is not a problem with topical therapy.

RECOMMENDED DOSAGE, DOSAGE SCHEDULE AND ROUTE OF ADMINISTRATION:

Usual adult and adolescent dose

Antifungal (vaginal) – Non-pregnant patients; Intravaginal, 500 mg as a single dose, preferably at bedtime; 200 mg once a day, preferably at bedtime, for three consecutive days; or 100 mg once a day, preferably at bedtime, for six or seven consecutive days.

Pregnant patients: Intravaginal, 100 mg once a day, preferably at bedtime, for seven consecutive days.

Note: The three-day regimen is not effective in pregnant women.